

SUPPLEMENTARY TABLE 1. Median number of acquired points for the initial rotor map with each mapping system.

	Rhythmia	Carto3	Ensite Precision *
Left atrium (number of maps = 17)	22135 [12603 – 30902]	2576 [2368 – 6019]	5452 - 6426
Right atrium (number of maps = 9)	11546 [10380 – 16678]	2372 - 2427	2317 - 3292

* Points in use. Total number of points was 37183 - 92480 for the left atrium, and 14296 - 19785 for the right atrium.

Data are shown as median [interquartile range] or as range, if only two maps were created.

SUPPLEMENTARY TABLE 2. Detailed procedural results in each patient.

Patient number	Number of rotors	Effect of rotor ablation	Number of sites with STD *	Effect of STD ablation	ATs ablated and other performed ablations (apart from focal ablation of rotors and sites with STD).
1	2	AT			- Small reentry in the posterior LA wall.
2	0 †				- Small reentry in the RSPV + right superior PVI.
3	1	SR			- Typical atrial flutter. - Right superior PVI.
4	1	AT			- Typical atrial flutter. - Focal AT in the posterior LA wall.
5	3	AT			- Perimitral flutter: ASL. - Typical atrial flutter. - SVC isolation + line between SVC and IVC to connect rotor ablation sites.
6	2	AT			- Small reentry in the LA roof. - Perimitral flutter: MAL. - Typical atrial flutter.
7	2	SR			- Macroreentry around the left pulmonary veins. - Macroreentry around the right pulmonary veins.

8	5	SR			- PVI (rotors within the RSPV and LSPV antra). - Focal AT in the crista terminalis.
9	3	AT			- Small reentry in posteroseptal mitral annulus. - Perimitral flutter: MAL. - Focal AT from proximal coronary sinus. - PVI (gaps from previous procedure). - Prophylactic ASL. - CTI ablation (gap from previous procedure).
10	0		2	AT	- Typical atrial flutter. - Macroreentry around the left pulmonary veins. - Right inferior PVI (gap from previous procedure).
11	0		4	No effect	- Right PVI (sites with SPT within the RSPV antrum). Left pulmonary veins were spontaneously isolated. - Line from anterior mitral annulus to scar in the anterior LA wall including site with SPT. - Empirical CTI ablation after electrical cardioversion.

12	1	SR			- Right PVI (rotor within the RSPV antrum). - Small reentry in the posterior LA roof.
13	1	No effect	2	AT	- Macroreentry around the right pulmonary veins. - Line from anterior mitral annulus to scar in the anterior LA wall including rotor. - Small reentry in the interatrial septum. - Macroreentry around the left pulmonary veins.
14	0		6	AT	- PVI (sites with SPT within the LIPV and RIPV antra). - Roof line (site with SPT in the LA roof). - Small reentry in anterior mitral annulus. - Small reentry in anteroseptal LA wall. - ASL to connect small-reentry focal ablations. - Prophylactic CTI ablation.

15	3	SR			- ASL to connect 2 focal rotors ablations (PVI performed in previous procedure). - Prophylactic CTI ablation.
16	1	SR			- Perimitral flutter: ASL. - Small reentry in anterior LA wall. - Left PVI (gaps from previous procedure). - Typical atrial flutter.
17	1	AT			- Right PVI (rotor within RSPV antrum). - Small reentry in LSPV: focal ablation. - Macroreentry around the left pulmonary veins: left PVI. - Perimitral flutter: ASL. - AVNRT.
18	0		3	AT	- PVI (sites with SPT within the LIPV and RIPV antra, and in the ridge). - Reentry around posterior LA wall scar. - Perimitral flutter: ASL. - Focal AT from the vein of Marshall. - Focal AT from the anterior LA wall. - Prophylactic posterior LA box.

ASL = anteroseptal line (line between mitral annulus and right superior pulmonary vein); AT = atrial tachycardia; AVNRT = atrioventricular nodal reentrant tachycardia; CTI = cavotricuspid isthmus; IVC = inferior vena cava; LA = left atrium; LIPV = left inferior pulmonary vein; LSPV = left superior pulmonary vein; MAL = modified anterior line (line between mitral annulus and left superior pulmonary vein); PVI = pulmonary vein isolation; RIPV = right inferior pulmonary vein; RSPV = right superior pulmonary vein; STD = spatiotemporal dispersion; SVC = superior vena cava.

* Sites with spatiotemporal dispersion plus non-continuous fractionation were only mapped if no rotors were present or if rotor ablation was ineffective.

† Spontaneous conversion from AF to stable AT occurred after rotor mapping in the left atrium was completed (no rotors were found).